

Nasogastric / Orogastric Tube

Gastric intubation via the nasal or oral route is used to decompress the stomach of air and/or gastric contents. This helps prevent aspiration and decreases intra-abdominal and intra-thoracic pressure.

Indications

- Anytime an adult patient in cardiac arrest is intubated

Contraindications

- Massive facial trauma
- Recent nasal surgery
- Suspicion of basilar skull fracture as evidenced by:
 - CSF otorrhea,
 - Battle's sign
 - Raccoon eyes
 - Mechanism of injury
- An orogastric tube may be considered if there is facial trauma, concern for facial fractures, or recent nasal surgery.

Relative contraindications

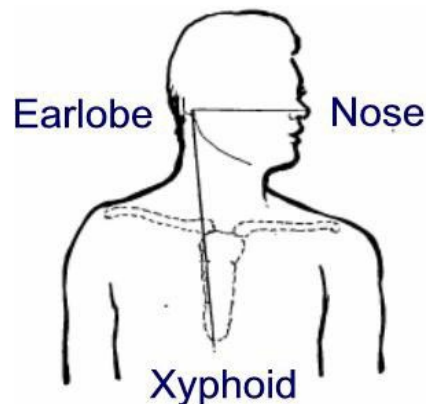
- Coagulation (bleeding) disorder
- Esophageal varices or stricture
- Recent esophageal procedure/surgery
- Alkaline ingestion

Equipment

- 1) NG/OG tube
- 2) Catheter tip irrigation 60ml syringe
- 3) Water-soluble lubricant, preferably 2% xylocaine jelly (if available)
- 4) Adhesive tape
- 5) Low powered suction device OR drainage bag
- 6) Stethoscope

Procedure for NG / OG tube insertion

- 1) Since patient will be intubated, the supine position will be the only option for patient positioning. However, if there is no concern for trauma, the neck may be placed in the sniffing position to aid in placement of the NG/OG tube.
- 2) Measure tubing from bridge of nose to earlobe, then to the xiphoid process. Mark the appropriate distance on the tube with pen or tape.



- 3) Lubricate tip of tube.
- 4) Tube placement
 - For nasogastric tube, select most patent/largest nares and pass lubricated tube in a posterior (not superior) direction. Advance tube parallel to nasal floor.
 - For orogastric tube, simply follow the curve of the tongue into the oropharynx (consider using tongue blade to aid in passing tube past tongue).
- 5) If resistance is met, gently attempt to corkscrew the tube slightly or remove and reattempt placement after confirming good patient positioning (may try other nare).
- 6) Withdraw tube immediately if changes occur in patient's respiratory status, drop in O₂ saturation below 95%, if tube coils in mouth, or if patient begins to cough, or becomes cyanotic.
- 7) Verify tube placement by listening over stomach while air is passed into tube by syringe, and then by examining aspirate when suction is applied.
- 8) Secure tube. May leave open to gravity or attached to intermittent low flow suction.
- 9) Monitor patient carefully.